

Front

(L)210mm

(W)148.5mm

pharmaniaga®



Dermal C

Cream

Betamethasone DP with Clotrimazole

DESCRIPTION

Smooth, white cream.

COMPOSITION

Each gram contains:
Betamethasone (as dipropionate) 0.05% w/w.
Clotrimazole 1% w/w.
Also contains Chlorocresol 0.10% w/w as preservative.

MECHANISM OF ACTION

Topical corticosteroids act as anti-inflammatory agents via multiple mechanisms to inhibit late phase allergic reactions including decreasing the density of mast cells, decreasing chemotaxis and activation of eosinophils, decreasing cytokine production by lymphocytes, monocytes, mast cells and eosinophils, and inhibiting the metabolism of arachidonic acid.

Clotrimazole acts against fungi by inhibiting ergosterol synthesis. Inhibition of ergosterol synthesis leads to structural and functional impairment of the cytoplasmic membrane.

PHARMACODYNAMICS

Topical steroid is a highly potent corticosteroid with local anti-inflammatory action, antipruritic, and vasoconstrictive properties.

Clotrimazole has a broad antimycotic spectrum of action in vitro and in vivo, which includes dermatophytes, yeasts, moulds, etc.

Under appropriate test conditions, the minimum inhibitory

concentration (MIC) values for these types of fungi are in the region of less than 0.062-4 (-8) mg/ml substrate. The mode of action of clotrimazole is primarily fungistatic. In-vitro activity is limited to proliferating fungal elements; fungal spores are only slightly sensitive.

In addition to its antimycotic action, clotrimazole also acts on *Trichomonas vaginalis*, gram-positive microorganisms (*Streptococci/Staphylococci*), and gram-negative microorganisms (*Bacteroides/Gardnerella vaginalis*).

In vitro clotrimazole inhibits the multiplication of *Corynebacteria* and gram-positive cocci-with the exception of *Enterococci* in concentrations of 0.5-10 mg/ml substrate and exerts a trichomonacidal action at 100 mg/ml.

Primary resistant variants of sensitive fungal species are very rare; the development of secondary resistance by sensitive fungi has so far only been observed in very isolated cases under therapeutic conditions.

PHARMACOKINETICS

Topical corticosteroids can be systemically absorbed from intact healthy skin. The extent of percutaneous absorption of topical corticosteroids is determined by many factors, including the vehicle and the integrity of the epidermal barrier. Occlusion, inflammation and/or other disease processes in the skin may also increase percutaneous absorption.

The use of pharmacodynamic endpoints for assessing the systemic exposure of topical corticosteroids is necessary because circulating levels are well below the level of detection.

Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. They are metabolised, primarily in the liver. Topical corticosteroids are excreted by the kidneys. In addition, some corticosteroids and their metabolites are also excreted in the bile.

Absorption of clotrimazole is less than 0.5% after application to skin/topical application.

INDICATIONS

Topical treatment of the following dermal infections : Tinea pedis, tinea cruris and tinea corporis due to *Trichophyton rubrum*, *Trichophyton mentagrophytes*, *Epidermophyton floccosum* and *Microsporum canis*; candidiasis due to *Candida albicans*.

CONTRAINDICATIONS

Hypersensitivity to corticosteroids, clotrimazole or other imidazoles, or to propylene glycol. Corticosteroid preparations are contraindicated in the treatment of herpes simplex, vaccinia or varicella, or tuberculous infections of the anal region (chronic fissure).

Dermal C Cream is contraindicated in facial rosacea, acne vulgaris, perioral dermatitis, napkin eruptions and bacterial or viral infections.

ADVERSE REACTIONS

The following adverse reactions have been reported infrequently with clotrimazole and betamethasone dipropionate when used in combination : Paraesthesia, maculopapular rash, oedema and secondary infection.

Reported adverse reactions to clotrimazole include erythema, stinging, blistering, peeling, oedema, pruritus, urticaria and general irritation of the skin.

The following local adverse reactions have been reported with the use of topical corticosteroids; burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae and miliaria, capillary fragility (ecchymoses), blurred vision and sensitisation.

Hypothalamic-pituitary adrenal (HPA) axis suppression, Cushing's syndrome and intracranial hypertension have been reported in children receiving topical corticosteroids. Manifestations of adrenal suppression in children include linear growth retardation, delayed weight gain, low plasma cortisol levels and absence of response to ACTH stimulation.

Manifestations of intracranial hypertension include bulging fontanelles, headaches and bilateral papilloedema.

PRECAUTIONS

The patient should be advised to use the medication for the full treatment time even though the symptoms may have improved.

Local and systemic toxicity is common especially following long continued use on large areas of damaged skin and in flexures. If used on the face, courses should be limited to 5 days.

This product should not be used with occlusive dressing as well as adhesive dressing.

Topical corticosteroids may be hazardous in psoriasis for a number of reasons including rebound relapses following the development of tolerance, risk of generalised pustular psoriasis and local and systemic toxicity due to impaired barrier function of the skin.

If irritation or sensitization develops with use, treatment should be discontinued and appropriate therapy instituted. In the presence of a bacterial infection, an appropriate antibacterial agent should be administered concomitantly. If a favourable response does not occur promptly, therapy should be discontinued until the infection has been controlled adequately. If there is a lack of response to the product, appropriate microbiological studies should be repeated to confirm the diagnosis and rule out other pathogens

attn

customer **Pharmaniaga Manufacturing Berhad**

date **10.09.2020**

1 colours

process colours:

Black

Please Chop & Sign For Approval

size **(L)210 x (W)148.5mm**

description **Dermal C Insert (2 sided Printing) - Front**

FocusPrint SDN BHD

t: 603-8766 6030 f: 603-8766 6033 (Factory)

website: www.focusprint.com.my

artwork prepared by: **cynthia yap**

email: graphic@focusprint.info (graphic Dept)

date:

ARTWORK LOG

Revision no.	Date	Reason for Change
01	04.07.19	- Amend wording.
02	04.04.2020	- Amend size to 210x148.5mm
03	09.04.2020	- Amend wording.
04	16.04.2020	- Amend wording.
05	30.04.2020	- Amend wording.
06	04.05.2020	- Amend wording.
07	10.09.2020	- Amend wording and revision date

IMPORTANT ! Please note that this colour print is for visual purpose only. Output colour might differ in actual production.

(L)210mm

Back

(W)148.5mm

before instituting another course of antimycotic therapy. Any of the side-effects that are reported following systemic use of corticosteroids, including adrenal suppression, manifestation of Cushing's syndrome, hyperglycaemia and glycosuria, may also occur with topical corticosteroids.

Systemic absorption of topical corticosteroids will be increased with the use of more potent corticosteroids agents, with prolonged usage or if extensive body surface areas are treated. Therefore, patients receiving large doses of potent topical corticosteroids, applied to a large surface area should be evaluated periodically for evidence of HPA axis suppression. If HPA axis suppression occurs, an attempt should be made to withdraw the drug to reduce the frequency of application, or to substitute with a less potent corticosteroid agent.

Recovery of HPA axis function is generally prompt and complete upon discontinuation of the drug. Infrequently, signs and symptoms of corticosteroid withdrawal may occur, requiring supplemental systemic corticotherapy.

This product is not for ophthalmic use.

Visual disturbance may be reported with systemic and topical (including, intranasal, inhaled and intraocular) corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes of visual disturbances which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR).

Use in children

Paediatric patients may demonstrate greater susceptibility to topical corticosteroid-induced hypothalamic-pituitary-adrenal (HPA) axis suppression and to exogenous corticosteroid effects in comparison to mature patients because of greater absorption due to large skin surface area to body weight ratio.

Long term continuous therapy should be avoided in all children irrespective of age. If used on children, courses should be limited to 5 days.

HPA axis suppression, Crushing's syndrome, linear growth retardation, delayed weight gain and intracranial hypertension have been reported in children receiving topical corticosteroids. Manifestations of adrenal suppression in children include low plasma cortisol levels and absence of response to ACTH stimulation. Manifestations of intracranial hypertension include a bulging fontanelle, headaches and bilateral papilloedema. The use of this product in diaper dermatitis is not recommended.

Dermal C cream contains

Cetostearyl alcohol which may cause localised skin reactions (e.g. contact dermatitis).
Propylene glycol which may cause skin irritation. Because

this medicine contains propylene glycol, do not use it on open wounds or large areas of broken or damaged skin (such as burns).

PREGNANCY AND LACTATION

Use in pregnancy

Clotrimazole has shown no teratogenic effect in animals but is foetotoxic at high oral doses.

Topical administration of corticosteroids to pregnant animals can cause abnormalities of foetal development including cleft palate and intra-uterine growth retardation. There may therefore be a very small risk of such effects in human foetus.

Since safety of topical corticosteroid use in pregnant women has not been established, drugs of this class should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus. Drugs of this class should not be used extensively in large amounts or for prolonged periods of time in pregnant patients.

Use in lactation

Since it is not known whether topical administration of corticosteroids can result in sufficient systemic absorption to produce detectable quantities in breast milk, a decision should be made to discontinue nursing or to discontinue the drug taking into account the importance of the drug to the mother. If used during lactation Dermal C Cream should not be applied to the breasts to avoid accidental ingestion by the infant.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Pharmaniaga Dermal C Cream has no influence on the ability to drive and use machines.

INTERACTION WITH OTHER MEDICAMENTS

Not known.

DOSAGE AND ADMINISTRATION

Gently massage sufficient product into the affected and surrounding skin areas twice a day, in the morning and evening, for 2 weeks in tinea cruris, tinea corporis and candidiasis and for 4 weeks in tinea pedis.

Duration of therapy: Clinical improvement, with relief of erythema and pruritus, usually occurs within the first 3 - 5 days of treatment. If a patient with tinea cruris, tinea corporis or candidiasis shows no clinical improvement after 1 week of treatment with this product, the diagnosis should be reviewed. In tinea pedis, the treatment should be applied for 2 weeks prior to making that decision. If the condition persists after 2 weeks in tinea cruris and tinea corporis and after 4 weeks in tinea pedis, treatment with this product should be discontinued. Alternate therapy may then be instituted with an appropriate antifungal agent only. The use of this product for more than 4 weeks is not recommended.

OVERDOSAGE

Acute overdosage with topical application of clotrimazole is unlikely and would not be expected to lead to a life-threatening situation.

Symptoms: Excessive or prolonged use of topical corticosteroids can suppress pituitary adrenal function, resulting in secondary adrenal insufficiency, and produce manifestations of hypercorticism, including Cushing's disease. Toxic effects are unlikely to occur following accidental ingestion of Dermal C cream.

Treatment: Appropriate symptomatic treatment is indicated. Acute hypercorticotoid symptoms are usually reversible. Treat electrolyte imbalance, if necessary. In case of chronic toxicity, slow withdrawal of corticosteroids is advised or by substituting a less potent corticosteroid because of the risk of glucocorticosteroid insufficiency.

Signs of toxicology appearing after such accidental ingestion should be treated symptomatically.

ROUTE OF ADMINISTRATION

Topical.

STORAGE CONDITIONS

Store below 30°C.
Keep container tightly closed.
Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

Tube of 15g.

Date of revision: 08th September 2020

PRODUCT REGISTRATION HOLDER/ MANUFACTURER:

PHARMANIAGA MANUFACTURING BERHAD
198001006232 (60016-D)
11A, Jalan P/1, Kawasan Perusahaan Bangi, 43650 Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia.

PRP 0327.5 080920

attn

customer **Pharmaniaga Manufacturing Berhad**date **10.09.2020****1** coloursprocess
colours:**Black****Please Chop & Sign For Approval**size **(L)210 x (W)148.5mm**description **Dermal C Insert (2 sided Printing) - Back** **FocusPrint** SDN BHD

t: 603-8766 6030 f: 603-8766 6033 (Factory)

website: www.focusprint.com.my

artwork prepared by: cynthia yap

email: graphic@focusprint.info (graphic Dept)**date:****ARTWORK LOG**

Revision no.	Date	Reason for Change
01	04.07.19	- Amend wording.
02	04.04.2020	- Amend size to 210x148.5mm
03	09.04.2020	- Amend wording.
04	16.04.2020	- Amend wording.
05	30.04.2020	- Amend wording.
06	04.05.2020	- Amend wording.
07	10.09.2020	- Amend wording and revision date

IMPORTANT ! Please note that this colour print is for visual purpose only. Output colour might differ in actual production.